

Adult Cardiac Arrest – Additional Treatments to Consider

Remember the H's and T's
Possible contributing factors to cardiac arrest

- Hypovolemia
- Hypoxia
- Hydrogen ion (acidosis)
- Hypokalemia/Hyperkalemia
- Hypoglycemia
- Hypothermia
- Toxins
- Tamponade (Cardiac)
- Tension Pneumothorax
- Thrombosis (Coronary or Pulmonary)
- Trauma

Consider possible causes of cardiac arrest and additional treatments:

- **Sodium Bicarbonate** 1 mEq/kg IO/IV
 - Prolonged arrest
 - Suspected hyperkalemia
 - Tricyclic antidepressant overdose (-tryptilines, etc.)
- **Narcan** 2-4 mg IO/IV
 - Suspected narcotic overdose
- **D10W** (D10) 25 g IO/IV
 - Repeat as necessary for persistent hypoglycemia
- **Albuterol MDI** (if available) – instilled directly into ET with Ambu Bag Ventilation
 - Airway Obstruction with asthma/COPD Hx
 - Suspected hyperkalemia
- **Calcium Chloride** 1 g IO/IV
 - History of dialysis or suspected hyperkalemia (rhabdomyolysis)
 - Suspected Calcium Channel Blocker overdose (-didipine, diltiazem, verapamil) and Beta Blocker overdose (-alol, etc)
- **Magnesium Sulfate** 2 g IO/IV slow IV push
 - Refractory VF
 - Torsades de Pointes
 - Severe asthma exacerbation
- **Ketamine** 2 mg/kg IO/IV (if available)
 - Severe asthma exacerbation
 - Status epilepticus refractory to benzodiazepines
- Needle decompression or finger thoracostomy of one or both sides of chest
 - Suspected tension pneumothorax
- NS/LR 1000mL – 2000mL IO/IV fluid bolus for suspected hypovolemia

~~In most cases, resuscitation of cardiac arrest in the wilderness is not successful. Please see termination protocol as deemed fit.~~

Exception: Drowning, Hypothermia, and Lightning Strike patients